

BMBR Guidance for Applicants

This guidance applies to BMBR funding opportunities that opened before 27 November 2024. For those that opened on or after 27 November 2024, please see the funding opportunity page for details on which guidance to follow.

These are the guidance notes for applicants who are applying for the Better Methods Better Research (BMBR) Programme. Applicants should submit an application via the REsearch Awards Lifecycle Management System (REALMS).

Applicants should consider these guidance notes for completing an application in conjunction with the additional information detailing the scope and remit of the BMBR Programme.

The 'Add New Ticket' button towards the top of the screen can be used to raise a support ticket if you have any questions relating to the call, or completion of the online application form.

Please regularly press the 'Save Draft' button, found towards the bottom of the screen to save your progress.

We would like to draw your attention to the 'Info still required' button at the bottom of the application page. We encourage you to click this well before submitting – it will bring up a list of all the sections that still need completing and any errors the system has detected (if any). These can be corrected in good time before the submission date.

Contracting organisation

Please give details of the organisation who will be the contractor if the project is funded. Please note that we expect the CI's contracting organisation (substantive employer) to act as the contractor. Your primary organisation will be shown by default. If this is not the contracting organisation, search for the correct contracting organisation by typing the name of the organisation in the search box. If the organisation you require does not appear in the search box, you can request to 'Add New Contracting Organisation'. New organisations need to be approved by the REALMS team prior to submission of your application. Please contact us if this request is being made within 2 weeks of the call close date. Please also bear in mind that:

- thought must be given to the most appropriate institution to act as the contractor as part of the application process, as changes are unlikely to be agreed once a funding decision has been made
- the contractor is expected to respond to annual financial reconciliation exercises, provide the final financial reconciliation statement for the project and to provide ad hoc requests for financial information during the lifetime of the project. In the unlikely event that a request is made for the contractor to differ from the CI's contracting (host) institution, the suggested contractor must be able to fulfil these expectations and to do so in the usual timeframes
- in the same way, the contractor is expected to respond to any queries relating to Intellectual Property, commercialisation and benefit realisation

If you have any questions, please contact nets-finance@nhr.ac.uk (mailto:nets-finance@nhr.ac.uk) before submitting your application.

Research title (Limit: 300 characters)

The project title should state clearly and concisely the proposed research. Any abbreviations should be spelled out in full.

Research type

Select the appropriate research type. If your proposed project includes any element of primary research, please select 'Primary Research'. If you are carrying out new analysis/synthesis of existing data, select 'Secondary Research' or 'Evidence Synthesis' as appropriate. If you are unsure which category to select you should choose the closest match to your project, as this can be adjusted later.

Start month

Note this will be from 1st of the month regardless of whether this is a working day or not. Please be realistic about your possible start date, taking account of the necessary contracting and staff recruitment prior to starting your project.

Start year

As above, please be realistic about your possible start date, taking account of the necessary contracting and staff recruitment prior to starting your project.

Research duration (months)

Ensure you include sufficient time to complete all aspects of the research, including applications for regulatory approvals (where required) and the final report.

End date

This field will automatically populate once you have saved the research duration information.

Total research costs

These figures are automatically populated from the details budget section.

Total National Health Service (NHS) support costs

These figures are automatically populated from the budget section. NHS support costs are the participant care costs which will not continue after the end of the study, and can usually be claimed in connection with NHS and non-NHS research.

Total NHS excess treatment costs

These figures are automatically populated from the budget section. NHS excess treatment costs are the additional costs or savings associated with the treatment of the participants during the research. The costs included are the additional costs compared with the current costs of standard care, which would continue to be incurred after the research, should the treatment become standard care in the future.

[Information is available on how to assess excess treatment costs](https://www.nihr.ac.uk/research-funding/application-support/costing-research)

(<https://www.nihr.ac.uk/research-funding/application-support/costing-research>).

Total Non-NHS Excess Treatment costs

These are similar to NHS excess treatment costs, but they mainly apply to Public Health and Social Care Research. They are unlikely to be applicable to BMBR research.

These figures are automatically populated from the detailed budget section. Non-NHS excess treatment costs include costs incurred in delivering the treatment, or intervention, which would continue to be incurred after the trial, should the intervention become standard care. The costs included are the additional costs compared with the cost of current standard care. Please note that NIHR has no provision to cover non-NHS excess treatment costs, and it is the responsibility of the applicant to secure these costs if they are needed. Where applicable, a letter from the provider of the treatment costs for the purposes of the study should be supplied.

Further information is available with the [AcoRD guidance](https://www.gov.uk/government/publications/guidance-on-attributing-the-costs-of-health-and-social-care-research)

(<https://www.gov.uk/government/publications/guidance-on-attributing-the-costs-of-health-and-social-care-research>) on how to allocate the costs of your proposal to each of the above categories.

Previous submission

Has this application, or a similar application, previously been submitted to this or any other funding body within the last 3 years?

We are keen to know if the application has been submitted to this or another NIHR programme or elsewhere, and you must be as open about this as possible. This includes, but is not limited to, any facts that, should they come to light at a future date, would embarrass either the programme or the individual who withheld the fact (e.g. if a member of the team holds a patent or has a financial interest within the research area).

Failure to disclose accurately or fully will be considered by the programme as academic misconduct and treated accordingly. You should also include in this section information on whether this or a similar application has been submitted to any programme previously, or to any other funder including other NIHR programmes. You should name and provide dates and outcomes of these. Please indicate whether you hold or have ever held an NIHR programme contract, which has been terminated prior to completion, extended in time or in terms of funding.

Select 'Yes' or 'No' to indicate whether this or a similar application has previously been submitted to this or any other funding body.

If Yes: use the 'Previous Submissions' button to select any relevant submissions from the list. If a previous submission was made to another funder or is not listed use the '+' button under the Other Previous Submissions section to record the details of that submission. For more information about resubmission of a research/trainee funding application, or joint funding please contact the appropriate NIHR research funding programme.

NIHR will not accept the same or substantially similar applications to more than one NIHR programme. If 2 similar applications are submitted, once the overlap is identified, the application that is most advanced through the funding process will continue and the second will not be taken further:

Similar applications will only be considered by 2 programmes concurrently if:

- the aims and research proposals are substantially different
- if successful, NIHR would be prepared to fund both proposals
- the successful delivery of one project is not dependent on the other

Research team

Lead applicant

Information on your name, main and other affiliated organisations and contact details will be automatically populated from your contact profile. Degrees and professional qualifications can be added to your contact profile. Update your contact profile to complete any missing mandatory information or make any updates here – you can do this via the 'My Profile' icon on your home page whilst in the application form by first clicking on the 'Update' button alongside your details in the Research Team section and then by clicking on the 'Update Contact Profile' button. Your contact profile will need to be validated prior to your application submission.

ORCiD ID

Please note: You and your Joint Lead/co-applicants are required to obtain a free unique ORCiD ID number and update your REALMS user contact profile with this before you can submit your application – the requirement for an ORCiD ID is mandatory for all applicants except PPI co-applicants. However, if a PPI co-applicant has an ORCiD account and wishes to add this to their REALMS contact profile they can. By clicking the link 'Create or connect your ORCiD ID' within the 'ORCiD' section of your user contact profile you will be taken to the ORCiD website where you will need to register or sign in. Once logged in to ORCiD and following acceptance of T&Cs, you will need to click on the 'Continue to import your ORCiD Data' button which will update your profile with your ORCiD ID, along with other associated ORCiD data (e.g. any publications and grants you have allowed ORCiD to make visible to the public or trusted parties). Grants and Publications drawn from ORCiD in this way will be available for you to use in populating the relevant sections of your application.

If additional publications and grants need to be added to an application, and you would like these available to select for other applications, you will need to update your ORCiD account first and then click 'Re-Import ORCiD Data' on your REALMS 'My Profile'.

Equality and diversity reporting system (EDRS)

The Research Team section lists any applicants who have been added to this application. Please note that the Joint Lead Applicant, Co-applicants and Administrative contacts are all required to be invited and accept their participation in a Stage 2 application (if not already invited and accepted at stage 1). The 'Update' button can be used to access the Applicant Details Form for each applicant and update the details of each user's involvement in this application. Different types of applicants can be added (e.g. Co-applicant/Co-applicant PPI).

Once ORCiD accounts have been linked up, the Lead, Joint Lead and co-applicants need to select which publications and grants are relevant for this application within the Applicant Details Form. This form can be found in the Action Items on each person's home page once the Lead applicant has added them.

The Applicant Details Form for all applicants must be in Submitted status in order to submit your application.

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the Lead applicant has added them.

The Applicant Details Form for all applicants must be in Submitted status in order to submit your application.

Research team - lead applicant 'update'

Click the 'Update' button to the right of your Lead Applicant information to add the information regarding your role on the application.

Role (Limit: 200 characters)

Specify Applicant's role in research. Enter the role that you will be undertaking in the research e.g. co-ordination and project management, analysis, methodological input etc.

% Full-time equivalent (FTE) commitment

This refers to the percentage of your time that you will commit to this project.

Organisation

Select your primary organisation for the purpose of this application. Your main organisation and any other affiliated organisations that you have already added will be listed by default. If the correct organisation is not listed, this can be added by using the 'Update contact profile' button towards the top of the page and adding a new organisation (to do this click on 'Other Organisation' in the left hand selection bar). Department – Select the primary department affiliated with. If you are a PPI representative and can't find or don't have an appropriate registered organisation, please start typing 'PPI Representative Base in...' and select the England/Scotland/Wales/Northern Ireland option as appropriate.

Department (Limit: 100 characters)

Select your department for the purpose of this application.

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appropriate registered organisation, please start typing 'PPI Representative Base in...' and select the England/Scotland/Wales/Northern Ireland option as appropriate.

Department (Limit: 100 characters)

Select your department for the purpose of this application.

Recent relevant publications

Provide details of a **maximum** of 6 of your most recent/relevant publications (in the last 10 years) relevant to this application.

To add relevant publications, click on the 'Open' button and either select the relevant listed publications pulled in from your ORCID record, and/or add any other new relevant publications. Once complete click 'Save selected Publications'.

Research grants held

This should include research grants held (as a named applicant) **currently** or **in the last 5 years** – as well as any additional previous grants, relevant to this application. Please include who the grant is with and the amount of each grant.

To add relevant research grants, click on the 'Open' button and either select the relevant listed research grant pulled in from your ORCID record, and/or add any other relevant research grant. Once complete click 'Save selected Publications'.

For each manually added research grant entry you should state the registration number, name of registry and the DOI of the main related publication.

Once all information has been entered and saved, click the 'Complete' button.

Joint lead applicant

Select 'Yes' or 'No' to indicate whether this application involves a Joint Lead Applicant, and where appropriate use the envelope icon to add a Joint Lead Applicant to your application and complete the necessary information. Once complete click the 'Invite' button. Once the

Joint Lead Applicant has accepted their invitation they will appear in the 'Research Team' list, and their information can be edited as necessary. The Joint Lead Applicant will need to complete their publications and grants information.

Co-applicants

Use the 'Add/Edit Co-Applicants' button to add a co-applicant(s) to your application and complete the necessary information. Once complete click the 'Invite' button. Once a co-applicant has accepted their invitation they will appear in the 'Research Team' list, and their information can be edited as necessary. Where appropriate co-applicants will need to complete their publications and grants information.

Do not include collaborators, who should be mentioned (if necessary) in the 'Detailed Research Plan' (upload document).

Co-applicants are those individuals with responsibility for the day to day management and delivery of the project. Co-applicants are considered part of the project team and are expected to share responsibility for its successful delivery. Collaborators normally provide specific expertise on particular aspects of the project but who do not share in the responsibility for the delivery of the project.

Patient and public involvement (PPI) co-applicants

We recognise and value the varied perspectives that members of the public, patients, service users and carers bring to a project as applicants and encourage their inclusion where appropriate. They are not required to provide a full curriculum vitae (CV) (i.e. N/A may be appropriate for 'Publication Record and Research Grants held'). PPI co-applicants should provide a summary of any relevant knowledge, skills and experience that they will draw upon to contribute to this project. This could include information about:

- previous or present work (paid or unpaid) with any relevant organisations
- links with any relevant groups, committees, networks or organisations

- experience of particular health conditions, treatments, use of services, being a carer – or as a member of a particular community
- knowledge and experience of research including previous research undertaken
- knowledge and experience of patient and public involvement including previous involvement activities
- skills from any other roles that are transferable Relevant qualifications, training and learning

For further information please access the ['Public Co-Applicants in Research' guidance](https://www.learningforinvolvement.org.uk/content/resource/public-co-applicants-in-research-guidance-on-roles-and-responsibilities/) (<https://www.learningforinvolvement.org.uk/content/resource/public-co-applicants-in-research-guidance-on-roles-and-responsibilities/>).

Signatories

Please note that signatories are no longer a requirement of a Stage 2 application submission. However, if your research proposal is recommended for funding you will be required to add the relevant signatories and shortly after receiving your funding recommendation outcome letter as part of the project start-up and contracting process (approximately 2-3 weeks after the funding assessment committee date).

Applicants must follow their host organisation's application review and sign off procedures before submitting an application. Failure to do this may result in any agreement to fund being withdrawn.

Administrative contact details

This facility allows you to provide an alternative contact(s) who will also have access to the application but will not be able to submit it when complete. The Lead Applicant must submit the completed application and will still receive all emails automatically generated through the system.

If you wish to include an administrative contact, use the envelope icon below to enter the administrative contact's details, and invite them to participate in this application.

This section contains the Scientific abstract and plain English summary.

Scientific abstract (Limit: 3,500 characters)

The scientific abstract should be a clear and concise scientific summary of the 'Detailed Research Plan/Methods', with a character limit of 3,500 characters (one side of A4 maximum). The following is a list of potential elements/headings that might be included depending on the design of the proposed research, the setting and programme being applied to, and whether it is for primary research or secondary research. It will be for researchers to decide the appropriate elements to be included in the scientific abstract, and could include elements outside this list. Applicants may find the [guidance on the EQUATOR Network website](https://www.equator-network.org/) (https://www.equator-network.org/) useful.

- research question
- background
- aims and objectives
- methods
- timelines for delivery
- anticipated impact and dissemination

Plain English summary of research (Limit: 3,500 characters)

A plain English summary is a clear explanation of your research.

Many reviewers use this summary to inform their review of your funding application. They include clinicians, other practitioners and researchers who do not have specialist knowledge of your field, as well as members of the public. If your application for funding is successful, the summary will be used on NIHR and other websites.

A good quality plain English summary providing an easy to read overview of your whole study will help:

- those carrying out the review (reviewers and funding committee members) to have a better understanding of your research proposal
- inform others about your research such as members of the public, health and social care professionals, policy makers and the media
- the research funders to publicise the research that they fund

If it is felt that your plain English summary is not clear and of a good quality, then you may be required to amend it prior to final funding approval.

It is helpful to involve patients/carers/service users/practitioners and members of the public in developing a plain English summary.

Content

When writing your summary consider including the following information where appropriate:

- aim(s) of the research
- background to the research
- design and methods used
- patient and public involvement
- dissemination

The plain English summary is not the same as a scientific abstract – please do not cut and paste this or other sections of your application form to create the plain English summary.

[Further guidance on writing a plain English summary](https://www.nihr.ac.uk/plain-english-summaries) (<https://www.nihr.ac.uk/plain-english-summaries>).

For additional support and advice on writing a plain English summary, please contact your local [Research Design Service](https://www.nihr.ac.uk/explore-nihr/support/research-support-service/) (https://www.nihr.ac.uk/explore-nihr/support/research-support-service/) (where applicable).

Patient and public involvement (PPI)

Please describe how patients/service users, carers and the public have been involved in developing this proposal (Limit: 3,500 characters)

You should describe who has been involved and why this is appropriate, what role(s) they have played, and what influence or change has happened as a result of their involvement.

Please describe the ways in which patients/ service users, carers and the public will be actively involved throughout the proposed research, including any training and support provided (Limit: 3,500 characters)

PPI Approach, management and support

Explain why your approach to patient and public involvement is appropriate for this proposal. In your description you will need to say who will be involved and why.

Please use this opportunity to describe how you plan to manage and coordinate the patient and public involvement activities in your project.

Describe how you will support and enable patients/service users, carers, the public and members of relevant communities to contribute to your research (e.g. access, payments, training).

We would also encourage you to outline plans for the capturing, evaluating and reporting the impact of patient and public involvement activities.

Patients, carers, service users and the public can be involved in every stage of a research project, from developing a proposal through to dissemination and evaluation. You can find further [resources to support the design of your PPI](https://www.nihr.ac.uk/ppi-patient-and-public-involvement-resources-applicants-nihr-research-programmes) (https://www.nihr.ac.uk/ppi-patient-and-public-involvement-resources-applicants-nihr-research-programmes).

PPI Lead

The role of the PPI lead can be undertaken by any of the co-applicants within the research team (or a named member of the team), who has the relevant skills, experience and authority to be accountable, represent, manage and embed patient and public involvement in all aspects of the research study/programme. This role should be a budgeted and resourced research team member. [View examples of activities a PPI lead might undertake \(https://www.nihr.ac.uk/role-patient-and-public-involvement-ppi-lead\)](https://www.nihr.ac.uk/role-patient-and-public-involvement-ppi-lead).

A summary of PPI activities

Please provide a summary of the proposed PPI activities embedded throughout the research project lifecycle. Please clearly signpost to other sections of the 'Detailed Research Plan' where the PPI is described further in relation to the relevant project stage e.g. dissemination, intervention design, data collection, analysis.

In rare cases where proposals do not involve patients/service users, carers and the public, clear justification must be provided. (Limit 3,500 characters)

Complete/justify as necessary.

Management and governance

Is Clinical Trials Authorisation required?

Select Yes/No as appropriate.

Does your project require ethics approval?

Select Yes/No as appropriate.

If yes, has ethics approval already been obtained?

Select Yes/No as appropriate.

Organisations for budget

Use the 'Add Organisations' and '+' button to select and add the organisations who will be responsible for budget items and add them to your application.

Application budget

This section should provide a breakdown of costs associated with undertaking the research as described in the proposal.

The information entered in this section should provide an analysis of the total funds requested to undertake the research proposed and should be based on current prices. These costs will be used to assess value for money.

- it is in the best interest to undertake a thorough, realistic and accurate costing. Where an outline/Stage 1 application has been produced and this is the full Stage 2 application, the Committee/Panel will pay close attention to any material increase in costs. You must provide a clear and full justification for all costs including NHS costs. You must also ensure that you include all costs including those required to secure good research management
- costs must be provided at current prices. An adjustment for inflation may be made during the life of the project at the discretion of the Department of Health and Social Care. Whilst allowances for incremental increases should be included on the form, nationally or locally agreed pay increase should be excluded
- years should be calculated starting from the anticipated start date of the proposed research. For example, if your research is expected to start on 01 June 2020 then its second year starts 01 June 2021
- when costs have been entered for year 1, it is possible to copy these costs to subsequent years, and they can then be edited if needed
- further itemisation of costs and methods of calculation may be requested to support the application at a later date

- payments will be made to the contracted organisation only and the contracted organisation will be responsible for passing on any money due to their partner organisations(s)
- payments are not able to be made in advance of need and, if funded, your payment will be made in accordance with your application budget. Please therefore ensure that your planned costs are included in the budget year in which they are most likely to be incurred
- appropriate sub-contracts must be put in place for any element of the research which is to be paid to another organisation
- NHS Support Costs, including costs for Social Care research are funded via Research Delivery Networks. Researchers should contact their local NHS R&D Department initially and, if they are unable to help directly or if there is no local NHS Research and Delivery Service (R&D) Department, contact the Regional Research Delivery Network (RRDN) Senior Manager for advice on NHS Support Costs. Further details about [RRDN contacts](https://www.nihr.ac.uk/support-and-services/support-for-delivering-research/research-delivery-network) (<https://www.nihr.ac.uk/support-and-services/support-for-delivering-research/research-delivery-network>) is available
- All applications are expected to have appropriate NHS, higher education institutions (HEIs), commercial and other partner organisation input into the finance section of the application form.
- Please note that whilst the applicable percentages will be used to calculate the maximum grant payable, the programme reserves the right to award a grant for less than this maximum where it is considered appropriate

Costs for un-named co-applicants

If you need to provide costs for more than one unnamed individual it is important to name them as 'TBA 1', 'TBA 2' etc. not just 'TBA', otherwise their costs will not appear in the PDF version of the form.

Contributions and joint funding

Contributions from other organisations and proposals for joint funding with another organisation can be recorded as such in the 'Contributions' tab. If you are considering applying for a joint funding arrangement please get in touch to discuss this.

Higher education institutions (HEIs)

Higher education institutions (HEIs) should determine the full economic cost (FEC) of their research using the transparent approach to costing (TRAC) methodology. For HEIs, up to 80% of FEC will be paid, provided that TRAC methodology has been used.

NHS Bodies and other providers of NHS services in England

For applications where the contractor is an NHS body or provider of NHS services in England, up to 100% direct costs will be paid.

Other partner organisations

- If you are a commercial organisation/consultancy, please fill in direct costs and commercial indirect costs. Indirect costs should be charged in proportion to the amount of research staff effort requested on the funding application form. Up to 100% of costs will be paid.
- If you are an 'other partner' organisation (e.g. charity or NGO), please fill in direct costs and other partner organisations indirect costs. Indirect costs should be charged in proportion to the amount of research staff effort requested on the funding application

form. Up to 100% of costs will be paid.

These are costs that are specific to the research, which will be charged at the amount actually spent and can be supported by an audit record. The should comprise:

Posts and salaries

This section presents an overview of salary and associated on-costs for the applicant(s) contributing to the research, include normal salary increments broken down individually.

Please include all members of staff working on the research by clicking 'add a new staff member' salaries' or editing a current one. If there are any applicant(s) whose costs are not being claimed you should still include their details within this section, but don't include any actual costs. Where applicants are already in receipt of NIHR funding for any part of their salaries (e.g. NIHR Fellowships), these should not be additionally charged to the project.

Where applicants are already receiving salaries funded by NIHR, these should be declared in the application by selecting 'Yes' in the 'Other NIHR Salary' drop-down list – please note that the NIHR will not fund more than 100% of an applicant's time across multiple NIHR projects/awards and this fact will be checked prior to contracting. If you are unsure about this requirement or have any questions please contact the appropriate NIHR research funding programme.

The Apprenticeship Levy can be included in the salary costs where relevant.

The annual costs of each applicant contributing to the research needs to be calculated by specifying the %FTE and number of months. Use current rates of pay and build in any known annual increments (again at current rates). You will not be able to claim for pay awards retrospectively, once your research is underway.

Please ensure that you check that the 'Cost Type' box reflects the organisation which correctly describes the employing organisation for a member of staff (e.g. HEI, NHS, Commercial, Other), as this impacts on the level of funding provided. Staff employed by a

Higher Education Institution (HEI) are funded at 80% of cost and staff employed by NHS, Commercial or other partner Organisations at up to 100% of cost.

Travel, subsistence and dissemination costs

This section includes journey costs, subsistence and dissemination costs, including conference fees. Where applicable, you will need to include the travel and subsistence costs of your Project Advisory Group, Steering Committee and/or Data Monitoring and Ethics Committee. Travel and subsistence costs relating to dissemination should also be included here, as should costs relating to overseas travel.

Journey costs – Enter the total costs of transport for all journeys for destination/purpose. If travel is by car, apply your institution's mileage rates (however this should not exceed HMRC approved mileage allowance payments, which is 45p per mile for the first 10,000 miles and 25p thereafter). Travel by the most economic means possible is encouraged. NIHR programmes do not usually fund first class travel.

Subsistence – Subsistence covers accommodation (if necessary) and meals associated with the travel, excluding any alcoholic beverages.

Dissemination costs

- **Conferences** – Where national or international conference costs are included, a statement naming the conference or purpose of travel and the benefit to the research must also be made; failure to adequately justify your attendance at a conference will mean the programme will not fund this cost. For research of up to 5 years, the programme will usually fund up to a maximum of 2 individuals to attend one international conference attendance, or one individual to attend 2 international conferences. For research beyond 5 years, the programme will usually fund up to a maximum of 2 international conference attendances per 5 years or part of 5-year research period

- Open Access Costs – Applicants should no longer include open access costs as part of their Stage 2 application Detailed Budget Costs

From the 1st of June 2022 all eligible awards contracts issued across [NIHR Programmes](https://www.nihr.ac.uk/researchers/i-need-help-funding-my-research/) (<https://www.nihr.ac.uk/researchers/i-need-help-funding-my-research/>), [NIHR Personal Awards](https://www.nihr.ac.uk/career-development/research-career-funding-programmes) (<https://www.nihr.ac.uk/career-development/research-career-funding-programmes>) and [NIHR Global Health Research Portfolio](https://www.nihr.ac.uk/research-funding/global-health) (<https://www.nihr.ac.uk/research-funding/global-health>) will have an Open Access Envelope allocated to them on top of the award value, which is ring-fenced for open access costs of peer reviewed research articles that arise directly from the research funded by the award in question. [Open Access funding guidance](https://www.nihr.ac.uk/nihr-open-access-publications-funding-guidance) (<https://www.nihr.ac.uk/nihr-open-access-publications-funding-guidance>).

- Other Dissemination Costs – Any large costs should be further detailed with a breakdown of constituent parts or a timescale profile of the costs. Meetings to share best practice, training events and events to disseminate research findings must be run at the lowest possible cost with minimal catering. 'Conferences' which are described as such are not eligible for funding.

Equipment

Essential items of equipment plus maintenance and related costs not included as part of estates should be input in this section. These can be lease or purchase costs. The purchase cost of pieces of equipment, valued up to £5,000 excluding VAT, will be considered.

Pieces of equipment costing more than £5,000 to purchase will usually need to be leased. Where applicants are leasing equipment with a purchase price of more than £5,000, a comparison of leasing versus purchasing costs must be provided in the 'Justification of Costs' section.

Items of equipment valued at £250 or more must be itemised separately; however, grouping same type equipment is permitted. Costs of computers are normally restricted to

a maximum of £1,000 each to include VAT and any associated software. If the purchasing organisation is unable to reclaim/recover the VAT on any piece of equipment, then it should be included in the stated cost. A statement of justification must be included in the relevant 'Justification of costs' section for any purchase above this limit.

Consumables

This section includes non-reusable items specific to the research. Please itemise and describe the requirements fully (e.g. postage, stationery, photocopying). These items should be research specific, not just general office costs which should be covered by indirect costs.

Patient and public involvement

Please itemise and describe fully the costs associated with Patient and Public Involvement. These are likely to include individual travel, out of pocket expenses, payment for time and any relevant training and support costs. Costs related to study participants should not be itemised here.

If voluntary, charity or community groups are supporting the research via activities such as facilitating contact with potential participants, contracting research activities or providing advice, an adequate budget must be included to compensate for their time and resources.

For more information on budgeting for involvement, please read the NIHR Payments Guidance for researchers and professionals.

Other direct costs

These are costs, not identified elsewhere, that are specifically attributed to the research. For example, costs associated with the use of research facilities, external consultancy costs, costs associated with inclusivity (which may include, but are not limited to justified translation of research participant material into other relevant languages), computing licensing, recruitment and advertising costs. Please note that for organisations claiming indirect/overhead costs, costs such as recruitment of staff, and general training (e.g. in common IT packages) are costs that should be covered by the indirect costs element of the

award being sought and should not appear in this section.

If external consultancy costs are included in this section they must be fully justified in the 'Justification fo Costs' section. Please specify the hourly rate and the number of hours and note that consultants must not be people who are already employed by the applicant's institution. If they are, any costs should be entered as direct costs in 'Posts and Salaries' section.

Indirect costs will be charged in proportion to the amount of research staff effort requested on the award. Commercial/Other Partner Organisations should calculate them, using their own cost rates.

They comprise:

- general office and basic laboratory consumables
- premises costs
- library services/learning resources
- Typing/secretarial
- Finance, personnel, public relations and departmental services
- Usage costs of major research facilities
- Central and distributed computing
- Charge out rates for shared equipment
- Cost of capital employed

NHS bodies or other providers of NHS services indirect costs

NHS indirect costs cannot be claimed through NIHR/DHSC programme funding. NHS Bodies or other providers of NHS services have been allocated [NIHR Research Capability Funding \(RCF\)](https://www.nihr.ac.uk/support-and-services/support-for-delivering-research/research-(RCF)) (<https://www.nihr.ac.uk/support-and-services/support-for-delivering-research/research->

[capability-funding/support-for-delivering-research/research-capability-funding](#)) to contribute to the cost of hosting NIHR/DHSC supported research.

HEI indirect costs

Total HEI indirect costs must be fully justified. HEIs are permitted to claim estate and other indirect costs. These costs are calculated on the basis of TRAC methodology. Proposals from other types of institutions/organisations should leave this section blank.

HEI indirect costs are based on the number of full-time equivalent research staff working on the research and the indirect/estates charges set by an institution. Please note HEI indirect costs cannot be claimed on shared staff costs. Where staff from more than one HEI are working on the research there may be different indirect/estates charges for each one. Please list each institution on a separate line. The applicant(s) should consult their HEI Finance Departments for the appropriate figures to include in the estate charges and other indirect cost sections.

Other partner organisation indirect costs

Other Partner Organisations can claim indirect costs which are the costs of resources used by the research that are shared by other activities. Please seek advice from your finance department about the appropriate cost for this section.

Other Partner Organisation indirect costs must be fully justified.

Research Support Costs

Please be aware that the research award does not include NHS or non-NHS support and/or treatment costs. These are funded via the Research Delivery Network (RDN). To access this funding, applicants will need to complete an [online SoECAT form](#) (<https://www.nihr.ac.uk/online-soecat-guidance>) via the [RDN's Central Portfolio Management \(CPMS\) system](#) (<https://www.nihr.ac.uk/getting-started-and-logging-cpms>).

Although patient care costs associated with the research are not funded through the research contract, this information still needs to be entered into NIHR application forms. An explanation of the costs should be provided in the written section of the application. This is because the Funding Committee will take NHS and non-NHS support and treatment costs into account when considering the value for money of the research.

It is essential that you consider these costs and discuss them with the relevant NHS body or non-NHS commissioner involved. This is to avoid any delay in commencing the research.

This section includes the following costs:

NHS Support Costs

These are the additional patient care costs associated with the research, which would end once the research activity in question has stopped.

If a patient care activity is primarily undertaken to facilitate research or is driven by the NHS duty of care to a patient, for example, to ensure the safety of a patient participating in research then it is attributed as a NHS support cost.

These might cover items such as:

- processing patient records
- obtaining informed consent
- extra patient tests
- extra in-patient days
- extra nursing attention

Further guidance can be sought from [RDN AcoRD specialists](https://rdn.nihr.ac.uk/lcrn/) (<https://rdn.nihr.ac.uk/lcrn/>) and via the [AcoRD Frequently Asked Questions](https://www.nihr.ac.uk/annex-b-dhsc-attributing-costs-health-and-social-care-research-development-acord-faqs) (<https://www.nihr.ac.uk/annex-b-dhsc-attributing-costs-health-and-social-care-research-development-acord-faqs>).

NHS Excess Treatment Costs

These are the patient care costs that would continue to be incurred if the patient care service in question continued to be provided after the research activity has stopped.

If an activity is integral to the provision of a treatment (or diagnostic) regime, whether this is standard or experimental, then it is attributed as an NHS treatment cost.

In determining NHS treatment costs you **must** assume that the patient care service being assessed will continue even though there may be no plans for it to do so.

Where patient care is being provided which differs from the normal, standard, treatment for that condition (either an experimental treatment or a service in a different location from where it would normally be given), the difference between the total treatment costs and the costs of the 'usual standard care' (if any) constitutes excess treatment cost/saving, but is nonetheless part of the treatment cost, not an NHS support or research cost. These costs should be determined in conjunction with your NHS body or provider of NHS services and their commissioners.

Please note if the patient care intervention under investigation is **in addition** to usual care there is no need to complete the 'Usual Treatment Costs' section, however this will need to be justified in the relevant 'Justification of Costs' section. If the patient care intervention under investigation either wholly or partially replaces usual care, the 'Usual Treatment Costs' section must be completed.

Further guidance can be sought from [RDN AcoRD specialists](https://rdn.nihr.ac.uk/lcrn/) (<https://rdn.nihr.ac.uk/lcrn/>) and via the [AcoRD Frequently Asked Questions](https://www.nihr.ac.uk/annex-b-dhsc-attributing-costs-health-and-social-care-research-development-acord-faqs) (<https://www.nihr.ac.uk/annex-b-dhsc-attributing-costs-health-and-social-care-research-development-acord-faqs>).

Non-NHS Support Costs

These are the additional costs associated with the research that would end once the research study in question had stopped, even if the associated treatment, intervention, or service continued to be provided. Social care and public health studies are eligible to receive RDN support.

Further guidance can be sought from [RDN AcoRD specialists](https://rdn.nihr.ac.uk/lcrn/) (https://rdn.nihr.ac.uk/lcrn/) and via the [AcoRD Frequently Asked Questions](https://www.nihr.ac.uk/annex-b-dhsc-attributing-costs-health-and-social-care-research-development-acord-faqs) (https://www.nihr.ac.uk/annex-b-dhsc-attributing-costs-health-and-social-care-research-development-acord-faqs). Please also refer to the [Public Health and Social Care researcher guidance](https://www.nihr.ac.uk/researcher-guidance-public-health-and-social-care-excess-treatment-costs-etcs) (https://www.nihr.ac.uk/researcher-guidance-public-health-and-social-care-excess-treatment-costs-etcs).

Non-NHS Excess Treatment Costs

These are the costs that would continue to be incurred if the treatment, intervention, or service in question continued to be provided after the research study stops.

The Funding Committee will take treatment, intervention or service costs into account when considering the value for money of the research. It is important that you consider these costs and discuss them with the NHS Trust, local authorities, third sector organisations or care/service providers involved in order to avoid any delay in commencing the research.

Typically treatment, intervention and service costs should be met by the commissioner, such as the Local Authority. Where it is not possible for the commissioner to pay the costs, social care research studies funded by NIHR may be eligible for intervention cost support from the DHSC. In order to apply for this, researchers will need to complete an additional [proforma](https://docs.google.com/document/d/1uDk_85D_6M5Lx5ILf4F-5HNCQ8vC9uqddWmdE_XyN1o/edit) (https://docs.google.com/document/d/1uDk_85D_6M5Lx5ILf4F-5HNCQ8vC9uqddWmdE_XyN1o/edit) as part of their Stage 2 application.

Further guidance can be sought from [RDN AcoRD specialists](https://rdn.nihr.ac.uk/lcrn/) (https://rdn.nihr.ac.uk/lcrn/) and via the [AcoRD Frequently Asked Questions](https://www.nihr.ac.uk/annex-b-dhsc-attributing-costs-health-and-social-care-research-development-acord-faqs) (https://www.nihr.ac.uk/annex-b-dhsc-attributing-costs-health-and-social-care-research-development-acord-faqs). Please also refer to the [Public Health and Social Care researchers guidance](https://www.nihr.ac.uk/researcher-guidance-public-health-and-social-care-excess-treatment-costs-etcs) (https://www.nihr.ac.uk/researcher-guidance-public-health-and-social-care-excess-treatment-costs-etcs).

When is a SoECAT not required?

Please note that, if there is no direct impact on participant care during the active award phase, completion of the SoECAT may not be necessary when applying for funding. This may be the case in the following circumstances:

- overarching programmes with no specific research study protocol
- infrastructure awards
- where the award is to be used solely for direct employment of a member of staff or
- purchase of an asset, and
- data or diagnostic reviews where recruitment data is not collected.

Such applications should be submitted with an explanation added to the 'Justification of Costs' section. Applicants should explain why they believe a SoECAT is not required for their submission.

If you are unsure whether this applies to you, further guidance can be sought from [RDN AcoRD specialists](https://rdn.nihr.ac.uk/lcrn/) (<https://rdn.nihr.ac.uk/lcrn/>).

- please provide a breakdown of research costs associated with undertaking the research and provide justification for the resources requested. This should include the following costs: staff costs, travel and subsistence, dissemination costs, equipment, consumables, patient and public involvement (PPI) and any other direct costs. For help with estimating PPI costs please see the NIHR Payments Guidance for researchers and professionals
- when justifying staff costs you should also provide the % amount of time input of each member of staff and link this to the specific area/work package of the proposed study where this input will be taking place
- you should indicate here how this research will potentially benefit the NHS and/or public health. For example, where appropriate, describe the likely costs savings or benefits in terms of numbers of patients treated, treatment times, service users or carers

supported etc.

- you should describe the value for money of the conduct of the proposed research
 - please provide a breakdown of the NHS costs associated with undertaking the research and provide justification for the resources required. If there are no NHS Support or Excess Treatment Costs associated with the research, you must explain why you think this is the case
 - please provide a breakdown of any non-NHS intervention excess treatment costs and provide justification for the resources required. Non-NHS intervention excess treatment costs should include costs incurred in delivering the intervention which would continue to be incurred after the trial, should the intervention become standard care. Please note that NIHR have no provision to cover non-NHS intervention excess treatment costs, and it is the responsibility of the applicant to secure these costs if they are needed
-
- Any additional, non-requested documents will not be considered by the funding committee during its review. However, there may be other requested documents e.g. cover letter, collaborative documents, dictated by the specification of the call.
 - To upload a document click on the icon next to the relevant upload type. All required uploads must be added for you to be able to submit your application.
 - Only the following document types can be uploaded: .doc, .docx & .pdf. If you upload a Word document the system will create a PDF version of it and both versions will be displayed below. Any other document types will need to be converted to PDF format before uploading. If you wish to remove or replace any specific documents all versions of that document will need to be deleted.

- **Attachment 1: Detailed Research Plan**

- It is mandatory to upload and submit a Detailed Research Plan (DRP), which is a full account of the proposed project. The DRP can include a Gantt chart described below in Section 7 and may contain relevant diagrams and charts within the 20 A4 pages.

- **Format**

- Your detailed research plan needs to be converted to a .PDF version before uploading, and should include the following:
 - text in arial font size 11
 - length not exceeding 20 A4 pages
 - a header containing your allocated project reference number, if known
 - a footer showing your page numbers
 - a Gantt chart
- Each recommended section of the Detailed Research Plan is not specifically word count limited, which provides applicants with the opportunity to use the 20 pages to provide elaboration in specific sections as required. Broadly, the Detailed Research Plan uploaded document for an intervention trial should follow the below format. For studies of diagnostic or prognostic tests, please modify these headings as appropriate.
- **1. Full title of project**
- **2. Summary of Research (abstract)**
- The scientific abstract should be a clear and concise scientific summary of the Detailed Research Plan/Methods. Applicants may wish to refer back to/or duplicate Section 6 of this form and have the opportunity to expand the information if needed.

- The following is a list of potential headings that might be included depending on the design of the proposed research. It will be for researchers to decide the appropriate elements to be included in this summary and could include elements outside this list.
- research question
- background
- aims and objectives
- methods, including justification of study design
- timelines for delivery
- anticipated impact and dissemination
- **3. Background and Rationale**
- Better Methods, Better Research (BMBR) funds the development or improvement of generalisable methods underpinning biomedical and health research. Please provide a clear explanation of the methodological issue to be addressed, how it is generalisable and how this will underpin and impact health research.
- Please briefly explain:
- Why is this research needed now?
- What is the knowledge gap this research will address?
- **4. Aims and objectives**
- Please summarise the aim of the research and research questions/objectives.
- Please provide:
- the aims(s) – broad and general statement(s) of intent addressing the overall purpose of your proposed research

- research objectives should be listed. They need to provide a focused indication of what you will do to achieve the aim(s)
- **5. Research plan/methods**
- **Research design:** Detail the design of your research ensuring it is clear how it builds on established research methods. Awareness of the current gaps in translation and uptake of methodology research should be demonstrated, and assurance provided that the project is designed to mitigate these risks to impact.
- **Statistical Analysis:** Clearly state the statistical test or software package to be used. Describe the methods and timing for assessing, recording and analysing outcomes.
- **Outcomes:** Provide detail of any outcome measures and how the proposed new method/improvement in a current method will be generally applicable without significant modification, beyond a single case study or research question.
- **Proposed deliverables:** Detail the deliverables from the study.
- **Value for money:** Provide an explanation of how the project offers value for money.
- Guidance for applicants on Equality, Diversity and Inclusion for study participants:
- The NIHR is committed to actively and openly supporting and promoting equality, diversity and inclusion in research. This is, in part, achieved by diversifying research participants in the studies we support and the voices of those who shape our research agenda by redesigning our processes, introducing targeted interventions and the effective monitoring and evaluation of impact.
- We invite you to consider how the above principles on equality, diversity and inclusion can be incorporated into your proposal.
- Helpful links:

- [Promoting equality, diversity and inclusion in research](https://www.nihr.ac.uk/about-us/our-key-priorities/equality-diversity-and-inclusion/) (https://www.nihr.ac.uk/about-us/our-key-priorities/equality-diversity-and-inclusion/)
- [The toolkit for increasing participation of ethnic minorities in health and care research](https://ethnicealthresearch.org.uk/research/resources-2/increasing-diversity-in-research-2/increasing-participation-of-black-asian-and-minority-ethnic-groups-in-health-and-social-care-research/) (https://ethnicealthresearch.org.uk/research/resources-2/increasing-diversity-in-research-2/increasing-participation-of-black-asian-and-minority-ethnic-groups-in-health-and-social-care-research/)
- [The INCLUDE Ethnicity Framework](https://www.trialforge.org/trial-diversity/include/) (https://www.trialforge.org/trial-diversity/include/)
- [The NIHR Race Equality Framework](https://www.nihr.ac.uk/nihr-race-equality-framework) (https://www.nihr.ac.uk/nihr-race-equality-framework)

- **6. Dissemination, Outputs and anticipated Impact**

- The purpose of this section is for the applicants to describe the planned outputs of the research, how these will be communicated and to whom, and how the research may lead to short – and longer-term – impacts. NIHR understands that the impact of any research may take time to be realised and will likely involve other funders, institutions and sustained efforts in practice. NIHR also recognises it may be difficult to provide definitive answers or guarantees on longer term impacts. However, applicants are invited to consider various aspects of pathways below and how the likelihood of impact can be maximised. This includes considering what outputs are produced, how these can be best connected to the health and/or social care environment, what efforts and investment are likely to be needed beyond the project, what barriers are likely to be encountered and what impacts the research is seeking to achieve.
- **a. What do you intend to produce from your research?**
- Please provide brief details of each anticipated output. Please note that the term 'outputs' refers to any tangible product of the research, not just academic publications.

Outputs can include, but are not limited to:

- conference presentation or other workshop events
- publications (academic or otherwise)
- guidelines (clinical, service or otherwise)
- other copyright (e.g. questionnaires, training aids, toolkits, manuals, software, etc.)
- new or improved design of medical devices or instrumentation
- new or improved diagnostic
- trial data that could be used to support a CE mark, market authorisation or equivalent
- trial data that could be used to shape or influence a healthcare market or government
- potential new drug or health or social care interventions
- **b. How will you inform and engage patients/service users, carers, NHS, social care organisations and the wider population about your work?**
- Describe who you need to communicate with within this research, and your plans for engaging relevant audiences. For impact, it is unlikely that simply making outputs available will be sufficient. Please consider, and outline the active approach you will take to engaging key parties, or identify the process you will use to identify them and formulate an engagement plan. Link to [NIHR Dissemination guidance: How to disseminate your research: Getting your message heard - and used](https://www.nihr.ac.uk/how-disseminate-your-research) (<https://www.nihr.ac.uk/how-disseminate-your-research>).
- **c. How will your outputs enter our health and care system or society as a whole?**
- Describe the process by which the research will enter the health and/or social care environment, including how your outputs will be acknowledged, selected and introduced for use in the health and care service or wider society. Where possible consider how the work will be able to adopted and implemented longer term. Please describe the

proposed route to market (commercial or non-commercial) for your outputs. Describe who is needed to take it forward and the relationship you currently (or propose to) have with these parties. If your outputs are likely to be commercially exploitable, please include details on how you plan to develop this.

- **d. What further funding or support will be required if this research is successful (e.g. from NIHR, other Government departments, charity or industry)?**
- Consider what investment or support may be needed at the end of this project to maximise impact. Not all projects will require this but if so, plans should be linked to the responses in points 2 and 3 above.
- **e. What are the possible barriers for further research, development, adoption and implementation?**
- Describe the difficulties which may be faced in generating impact from your research. These may be difficulties you will face yourself, or challenges faced by those in the implementing context (e.g. clinicians). How will you ensure wider implementation or learning from project outputs, including approaches for benchmarking (method performance and implementation costs), outreach and the ability to influence wider audiences?
- will the proposed research use data, technology, materials or other interventions that are subject to any form of intellectual property protection (e.g. copyright, design rights, patents) or right owned by another organisation(s)? If yes, provide brief details including how such third party IP will be accessed (e.g. collaboration agreement, drug supply agreement)
- what are the key current and future barriers to uptake of any likely output or innovation directly in the health and care service, through commercial exploitation or other means,

e.g. potential regulatory hurdles?

- what are the challenges for getting your research implemented in terms of acceptability, accessibility and feasibility? How will you address these?
- **f. What do you think the impact of your research will be and for whom?**
- Describe the impacts you aim to achieve as a direct result of the project and those which are anticipated longer term. Please consider how any smaller, more immediate effects may mature over time into larger scale or more significant effects, and the steps by which this may be achieved. As far as possible, indicate anticipated timescales for these benefits and a quantitative estimate of their scale. Impacts may include, but are not restricted to:
 - patient/service user/carers benefit
 - healthcare/social care staff benefits
 - changes in NHS or care services (including efficiency savings)
 - commercial return (which could contribute to economic growth)
 - public wellbeing
- **g. If applicable, how will you share with study participants the progress and findings of your research?**
- What strategies will you use to keep your research participants informed of the progress of your project and the findings? Consider the ethical implications of informing study participants and also what the most accessible methods could be, such as newsletter, leaflets, webpages, social media and where relevant different languages and formats. The Health Research Authority provides [guidance on the information participants of trials should receive at the end of the study](https://www.hra.nhs.uk/planning-and-improving-research/best-practice/publication-and-participants-of-trials-should-receive-at-the-end-of-the-study) (<https://www.hra.nhs.uk/planning-and-improving-research/best-practice/publication-and->

[dissemination-research-findings/](#)).

- **7. Projects/research timetable**

- **Project Timetable:** Provide a project timetable with key deliverables and milestones. A Gantt diagram, included as a visual summary of the overall project, is considered helpful.
- **NIHR Monitoring:** Your progress will be regularly monitored, comparing your submitted progress reports against your planned deliverables and milestones checked. These progress reports will be based on the project timetable and milestones and will occur at approximately 6 month intervals. If you are late producing progress reports and a single draft final report of the expected standard for the BMBR Programme, we reserve the right to withhold payments as per the contract. Applicants should note that the BMBR Programme monitors the degree to which requested timetables are met, and that having a proven track record in delivering on time may be a consideration when deciding future awards.
- If your project requires ethics/regulatory approval, you should allow time for obtaining these approvals and submitting appropriate documents to the BMBR Programme. Time for the recruitment of staff, the production of the draft report and a draft paper suitable for publication in a peer-reviewed journal should also be included.

- **8. Project management**

- All project proposals should include details of how the project will be managed.

- **9. Ethics/Regulatory Approvals**

- Outline any ethical and/or other regulatory issues, and arrangements for handling them. Consider when the project requires approval by an ethics committee and/or any

other regulatory body.

- Guidance on the application process for ethical and other approvals can be found on the [HRA website](https://www.hra.nhs.uk/approvals-amendments/what-approvals-do-i-need/) (<https://www.hra.nhs.uk/approvals-amendments/what-approvals-do-i-need/>). Please note that if your study is led from England and involves the NHS in England you should apply for [HRA approval](https://www.hra.nhs.uk/approvals-amendments/what-approvals-do-i-need/hra-approval/) (<https://www.hra.nhs.uk/approvals-amendments/what-approvals-do-i-need/hra-approval/>).
- If you are using patient information from an existing database, you should check whether the patients have given their consent for their data to be included in that database for research purposes, or if not whether the database is exempt under Section 251 of the NHS Act 2006. Where exemptions are not already in place, approval to use confidential patient information without consent must be requested from the HRA who make decisions with advice from the [Confidentiality Advisory Group \(CAG\)](https://www.hra.nhs.uk/about-us/committees-and-services/confidentiality-advisory-group/) (<https://www.hra.nhs.uk/about-us/committees-and-services/confidentiality-advisory-group/>).
- Researchers may find the [SPIRIT 2013 Statement](https://spirit-statement.org/) (<https://spirit-statement.org/>) a useful resource when preparing their protocol for ethics and other approvals.
- **10. Project/research expertise**
- Outline the particular contribution each member of the team will make towards the project and the particular contribution that collaborators are intending to make. In addition, give details of supervision arrangements for junior staff involved, and details of arrangements to retain critical staff or co-applicants who do not have a contract employment that spans the project period. The BMBR Programme recommends teams proposing randomised controlled trials to include input from an accredited clinical trials unit, or organisations with equivalent experience.

- If commercial partners are involved in the study, a detailed description of the contributions and expectations from all parties must be included.
- PPI Lead
- There should be a named person with appropriate skills and experience who is responsible for leading the PPI element within the project. This role should be an adequately costed and resourced research team member who is able to manage the PPI plans and related activities. [Examples of the activities a PPI lead might undertake.](https://www.nihr.ac.uk/role-patient-and-public-involvement-ppi-lead)
(<https://www.nihr.ac.uk/role-patient-and-public-involvement-ppi-lead>)
- **11. Success criteria and barriers to proposed work**
- Please set out the measurements of success you intend to use, the risks to the proposed research and how you intend to mitigate against them.
- **Upload 2: CTU support letter (if required/appropriate to the study). Not a mandatory upload**
- If appropriate to the study, please supply and upload a Clinical Trials Unit (CTU) letter of support.
- **Upload 3: References (maximum 3 pages of A4)**
- List all references cited in the full project description, using either the Vancouver or Harvard referencing conventions, and include the DOI of the main related publication.
- **Upload 4: Schedule of Events Cost Attribution Tool (SoECAT), if applicable**
- A completed Schedule of Events Cost Attribution Tool (SoECAT), is now required to be uploaded and submitted for projects that are to be registered with the NIHR portfolio. A SoECAT is required by NIHR and NIHR non-commercial partner researcher funders for studies involving participants under an NHS or Health and Social Care duty of care.

- The exception to this is where no participants are being recruited and there is no Excess Treatment Cost or NHS Support Cost support, for example in the case of some stand-alone methodological studies. In this case, a SoECAT will not be required. This should be confirmed by an AcoRD Specialist in writing.

Potential conflicts

Please declare any conflicts or potential conflicts of interest that you or your co-applicants may have in undertaking this research, including any relevant, non-personal & commercial interest that could be perceived as a conflict of interest. If in doubt, you should err on the side of disclosure.

Agreement to terms and conditions

I have read and understood the terms on which I have been nominated as lead applicant for this proposal along with the associated documentation and accept this role.

A list of full terms and conditions can be found here: [terms and conditions](#)

(<https://www.nihr.ac.uk/terms-and-conditions-use-netscc-misrealms>).

Checklist of information to include when submitting a NIHR Stage 2 research application

As a guide, applicants should ensure they have included the below necessary information prior to submitting their application:

- appropriate and relevant involvement of [patients/service user, carers and the public](#) (<https://www.nihr.ac.uk/ppi-patient-and-public-involvement-resources-applicants-nihr-research-programmes>)
- a good quality [Plain English Summary](#) (<https://www.nihr.ac.uk/plain-english-summaries>)

- a clear description of team member roles and contribution
- a clear scientific abstract
- a full and accurate detailed budget breakdown
- a clear justification of costs/value for money

Document uploads

- a clear Detailed Research Plan outlining the study design, methods, dissemination etc.
- aCTU letter of support (if applicable, or a justification for why this is not required).
- references, maximum 3 pages of A4
- a completed approved SoECAT (if applicable, see Detailed Budget section above)
- letters of support from collaborators/partners (where relevant)
- any Papers In Press (where relevant)